

WEBINAR COMPANION REPORT • OCTOBER 2026

Crisis to stability

Measuring the client journey: five tools for seeing whether people move, across programs and over time.

Crisis

safety and basic needs

Stabilization

supports in place

Recovery

gains that hold

Dr. Alina Turner, HelpSeeker Technologies

Based on the webinar of October 6, 2026, with the published evidence behind each tool

WHY THIS MATTERS

Programs report moments. People live movement.

Every program in a community can report success while nobody can say whether the person it served is more stable than a year ago. The difference is not effort. It is what gets counted.

One person, eighteen months, four agencies. A newcomer family's father loses his job in winter.

January • crisis line and food bank 3 calls, 4 hampers

Spring • counselling, another agency completed

Summer • employment program placed

Next winter • settlement and housing goals met

Four intake forms, four funders, four accurate reports. Not one of them can say whether his family's life stabilized, or what it took to get there.

COUNTED BY PROGRAM

86%

average completion across four programs

Crisis services  88%

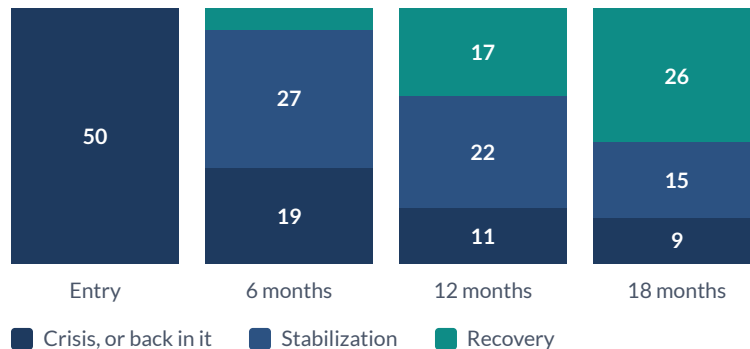
Counselling  81%

Employment  84%

Settlement  90%

Every funder report reads as a success.

COUNTED BY PERSON: THE SAME 50 PEOPLE OVER 18 MONTHS



52%

reached recovery by 18 months

18%

back in crisis, or never left it

40 days

average with no program attached between handoffs

Neither view is wrong. They answer different questions. Only the second tells you who is stuck, where, and for how long.

Illustrative teaching figures, not drawn from any organization's data. Published rates for the same patterns follow on pages 5 and 8.

Outputs, outcomes and impact are different questions

Most reporting stops at the first rung. The person's journey only becomes visible at the third, and only if someone keeps looking after the file closes.

1

Output

What we did. Sessions held, referrals made, hampers given, hours of support.

Measured by counting activity.

2

Outcome

What changed while we were involved. A lease signed, a job started, fewer crisis calls this quarter.

Measured by comparing entry and exit, inside one program.

3

Impact

What changed and lasted. Still housed, still working, still connected a year after every program closed.

Measured by following the person after discharge, across agencies.

The practical line

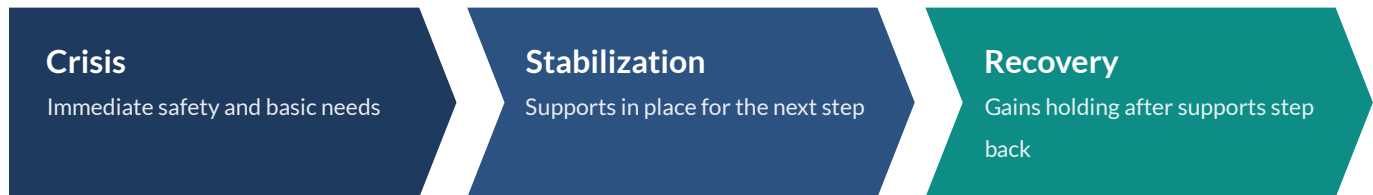
You cannot see impact from inside one program. In the data, impact is an outcome whose measurement date sits after the case closed. That is why journey measurement is how outcome data becomes impact evidence.

This reading matches the major evaluation standards. The OECD defines outputs as within the implementing team's control, outcomes as the short and medium-term effects of those outputs, and impacts as the longer-term changes that follow. The W.K. Kellogg Foundation's logic model guide notes that impact "often occurs after the conclusion of project funding." Canada's Treasury Board does not use the word impact at all; its top tier is the result a department seeks to influence but does not control.

OECD (2023), Glossary of key terms in evaluation and results based management, 2nd ed., pp. 36, 45 · Treasury Board of Canada Secretariat, Policy on Results, Appendix A · W.K. Kellogg Foundation (2004), Logic model development guide, p. 2

Three phases, the moves between them, and where people get lost

A phase is a judgement made from recorded signals, not a box someone ticks from memory.
Agree the signals once and the phase follows from the data.



SIGNALS YOU CAN RECORD

Emergency or crisis contact, shelter or hospital stay, urgent referral, safety plan opened

SIGNALS YOU CAN RECORD

Enrolled in ongoing support, income in place, goals set, regular contact

SIGNALS YOU CAN RECORD

No crisis contact in the period, goals holding at follow-up, the person's own rating stable

Count the moves, not just the states. Movement is the change between phases: moving forward, holding, and returning. Returns to crisis are a normal part of recovery in every stage model we reviewed, so count them rather than hide them. Record where people go when they leave, including "unknown". If unknown is your biggest exit category, that is your first finding.

THE TRANSITION IS THE DANGEROUS MOMENT

12.7×

higher risk of death than the general population in the first two weeks after release from prison (Washington State cohort, n = 30,237)

1,132 vs 484

suicides per 100,000 person-years in the three months after psychiatric discharge, against the longer-run rate (meta-analysis)

-0.20

pooled effect of Critical Time Intervention, a time-limited bridge across the transition, measured on homelessness after shelter or hospital discharge (8 studies). It reduces risk; it does not end it.

The handoff

The weeks between one discharge and the next intake. No program owns them, so no program measures them.

The quiet return

A step back toward crisis at another agency reads as a brand new client. The system learns nothing.

After discharge

Where the outcome is decided, and where reporting usually stops.

Binswanger et al. (2007) • Chung et al. (2017) • Tsai, Schrader and Beydoun (2025) • Prochaska, DiClemente and Norcross (1992) • Andresen, Oades and Caputi (2003)

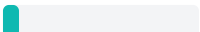
Transitions between services are documented hazards

Across corrections, psychiatric care and hospitals, the weeks after leaving one service are measurably more dangerous than the months that follow. This is the best-evidenced finding behind the deck.

Supported

Death after release from prison, first two weeks

First two weeks  12.7×

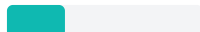
General population  1×

Washington State cohort, 30,237 people. Binswanger et al. (2007).

Supported

Overdose deaths after release, British Columbia

First two weeks  38.8

Overall after release  11.2

Per 1,000 person-years. Kinner, Gan and Slaunwhite (2021). In Ontario, 20 percent of post-release drug toxicity deaths came in the first week (Groot et al., 2016).

Proven

Suicide after psychiatric discharge

First three months  1,132

Overall after discharge  484

Per 100,000 person-years. Systematic review and meta-analysis, Chung et al. (2017).

Supported

Readmitted to hospital within 30 days, Toronto

Adults experiencing homelessness  22.2%

Matched low-income controls  7.0%

Matched cohort. Saab, Nisenbaum, Dhalla and Hwang (2016).

AND FOLLOW-UP AFTER DISCHARGE IS UNEVEN IN CANADA

70%

of Canadian hospital stays for mental health or substance use were followed by a physician visit within 30 days, 2023 to 2024. Three in ten were not.

CIHI (2025)

14.2%

of Ontarians leaving a first hospitalization for schizophrenia saw a psychiatrist within 7 days; 49.2 percent saw a physician within 30.

Rotenberg et al. (2023)

0

published measures of the gap between leaving one community program and starting the next. Nobody owns the handoff, so nobody counts it.

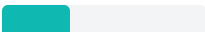
Evidence review, October 2026

Bridging support across a transition reduces harm

One model of help at the handoff has randomized-trial evidence behind it: Critical Time Intervention, a time-limited worker who carries the person from one service into the next. It was tested after shelter and psychiatric hospital discharge. The effects are real and modest. Say "reduces", not "ends".

Proven, one model

Homeless nights over 18 months, first randomized trial

Usual services		91
Critical Time Intervention		30

Susser et al. (1997).

Proven, one model

What the later trials and reviews found

Odds of homelessness at 18 months	0.22 (0.06 to 0.88)
Odds of psychiatric rehospitalization	0.11
Pooled effect on homelessness, 8 studies	SMD -0.20
Intensive case management, days homeless	SMD -0.22
Fewer people lost from care	RR 0.68

Herman et al. (2011); Tomita and Herman (2012); Tsai, Schrader and Beydoun (2025); Ponka et al. (2020); Dieterich et al. (2017), Cochrane.

THE WARM HANDOFF: PROMISING, NOT PROVEN

Contested

Attending the first appointment after a warm handoff

Largest study, 2,690 referrals, urban safety-net hospital **OR 0.96 (0.79 to 1.18)**

Pediatric primary care, 328 referrals **OR 3.30**

Brief introduction vs none, 171 referrals **80.3% vs 57.1%**

Pace et al. (2018); Anand and Desai (2023); Tobin et al. (2026); Readdean et al. (2026). All observational, none in community social services.

The evidence on how to hand off is mixed. The evidence that the handoff is dangerous is not.

Measure the gap first. Then test what closes it in your own services.

Five measures, each tight enough to calculate

You probably hold most of the data for these already. What is usually missing is the join across programs, and one follow-up after the file closes.

Time to stabilization

Days from the first crisis contact to the first enrolment in ongoing support.

Mirrors length-of-time measures in national system performance standards.

Practice standard

Handoff gap

Days between exit from one program and entry to the next, for the same person.

No published measure defines it. That absence is the point: nobody owns the gap, so nobody counts it.

HelpSeeker practice

Return to crisis

Share of people who reached stabilization and had a crisis contact within 6 or 12 months.

6, 12 and 24 month return windows are standard in system performance measurement.

Practice standard

Recovery hold

Share of people in recovery at discharge who are still there at the 6 and 12 month follow-up.

The closest practical proxy for impact. Trials show durable gains are measurable over years.

Proven construct

Self-rated progress

Change in the person's own goal ratings, by domain, from intake to each review.

Individualized goal measures are reliable and sensitive to change (see pages 13 and 14).

Supported

Every measure needs two things

A date on each event, and a person ID that matches across services. Without them none of the five can be calculated, which is why the data foundation matters more than any dashboard.

Grades from the evidence review behind this report: Proven means randomized trials or systematic reviews; Supported means observational or linked administrative data; Practice standard means a recognized government or sector standard; HelpSeeker practice means our experience, with no external source found.

Returns, unknown exits and lasting gains can all be measured

Housing is the sector with the most published journey data, so it shows what the five measures look like at scale. The same logic applies to settlement, employment, mental health or youth services; the data there is simply thinner.

Practice standard • Supported values

Return to crisis: published return rates

US, within 24 months of an exit to permanent housing  16.6 to 19.7%

Canada, back in shelter within a year, all leavers  22.1 to 24.3%

HUD (2025), FY2020 to FY2024; HICC (2025), six cohorts in BC, Ontario and Nova Scotia. Different denominators, same lesson.

Supported

Exits to unknown

53.7 to 63.3%

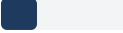
of people leaving shelters each year from 2018-19 to 2023-24 had no recorded reason for exit. Only 34 percent were recorded as housed, a figure HICC itself calls an underestimate.

HICC (2025). If unknown is your biggest exit category, that is your first finding.

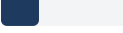
RECOVERY HOLD: HOUSING WITH SUPPORT LASTS FOR YEARS

Proven

Time stably housed, 24 months, Housing First with ACT  71%

Usual care  29%

Housed all of the last six months, Housing First  62%

Usual care  31%

At Home/Chez Soi, high-needs participants. Aubry et al. (2016); Goering et al. (2014).

Proven

Days stably housed in year six, Toronto, high needs 85.5% vs 60.3%

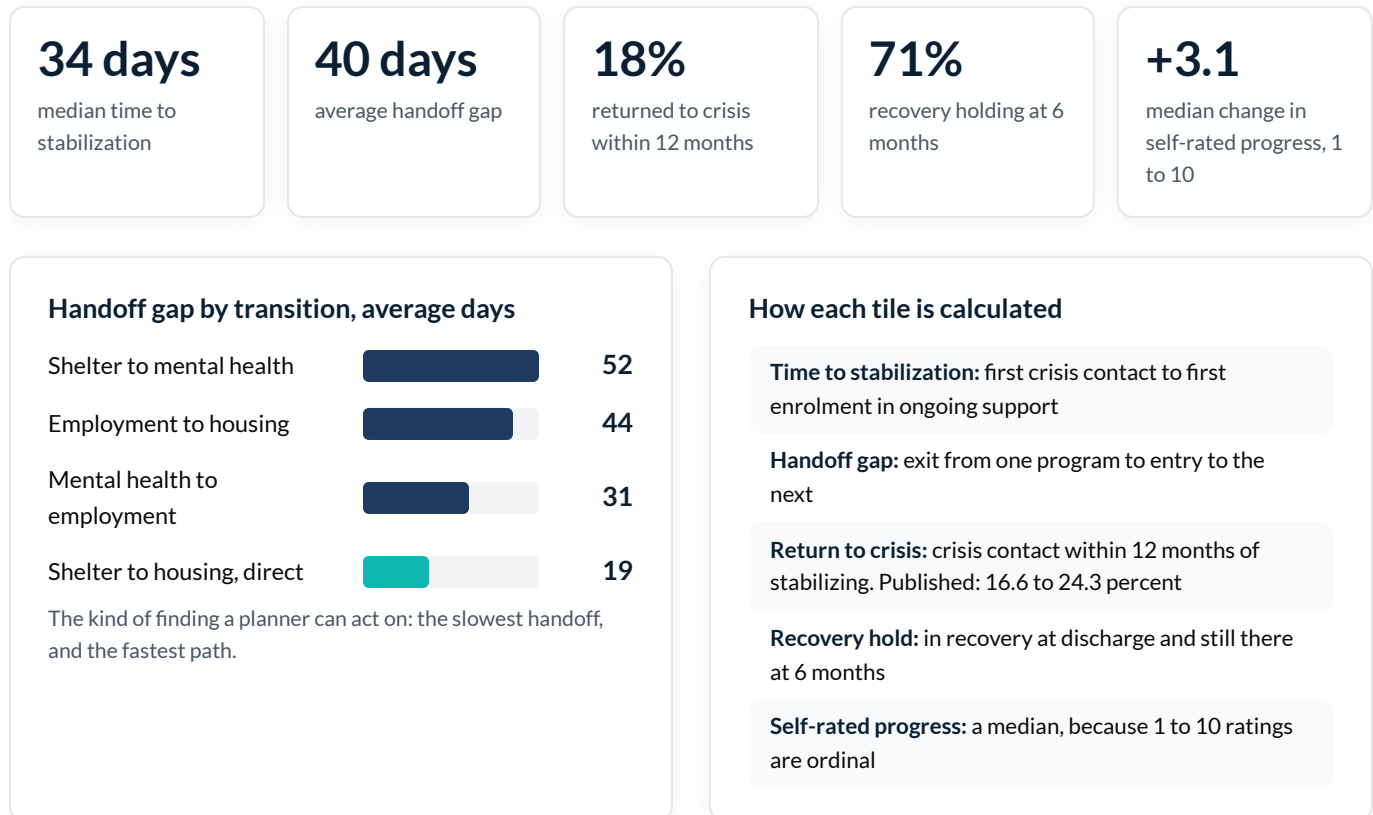
Days to housing, high needs (time to stabilization) 73 vs 220

Pooled rate ratio for stable housing, systematic review 1.42

Stergiopoulos et al. (2019); Aubry et al. (2016); Aubry et al. (2020). No trial measures a "recovery phase" directly, so page 2's 52 percent stays illustrative.

A journey dashboard for a network of agencies

A design for a coordinated network, reading the same people across every program they touched. The figures are illustrative; the definitions are the point.



Design of a Mareto journey dashboard. Illustrative figures for a different network than page 2's 50 people, not drawn from any organization's data.

One person, ten fields, shared definitions

Matching the same person across services is solved science when the identifiers are there. The work is collecting them consistently, and agreeing what the words mean.

95.87%

of records linked by Statistics Canada using names, date of birth, sex, postal code and municipality, rather than a name alone

99.36%

specificity in the same linkage, meaning very few false matches. Service records with fewer identifiers will link at lower rates.

THE MINIMUM JOURNEY DATASET

- **Person ID** matched on more than a name: date of birth, phone, prior address.
- **Program and agency** for every step.
- **Start and end dates**, which turn enrolments into a timeline with visible gaps.
- **Referred from and to**, which shows the handoff rather than two separate files.
- **Exit destination**, including "unknown", counted honestly.
- **Status on a shared definition** agreed with partners.
- **Phase at entry and exit**, derived from agreed signals. New
- **Consent scope and date**, recorded as a history. New
- **Goals and self-ratings** in the person's own words.
- **Follow-up date and result**, the only way to see recovery hold. New

Then agree what the words mean, with partners, before you add anything up (page 11).

Statistics Canada (2018), Canadian Cancer Treatment Linkage Project • Setoguchi et al. (2014) • Canadian Observatory on Homelessness (2012, revised 2017). Most of the first seven fields already exist in common case management and homelessness information systems; phase, consent history and follow-up result are usually additions.

Recognize the same person, and agree what the words mean

Two habits make every other measure possible: catch the duplicate at the door, and stop adding up numbers that count different things.

A duplicate check at a partner's intake

An employment service starts a new intake. Before saving, the system finds an existing person who matches on date of birth, phone and prior address, not only the name.

Shelter Jan to Mar

Mental health Apr, ongoing

Employment linked as program three

Three agencies so far, one person. The employment service did not create a fourth stranger.

95% • 98%

sensitivity and specificity from two or three indirect identifiers plus the provider's ID (Setoguchi et al., 2014)

97.74%

sensitivity in Statistics Canada's linkage on six identifiers (2018). Records with fewer identifiers link at lower rates.

Matching is a trust question as much as a technical one. Explain why records connect, and let people say no. Public support for linking data depends on seeing the public benefit (Aitken et al., 2016; Teng et al., 2019).

DO NOT INVENT DEFINITIONS. ADOPT ONE THAT EXISTS.

"Housed" means three things

Agency A counts a shelter bed. Agency B counts a lease. Agency C counts couch surfing with family. Add the totals together and the number means nothing. The same happens with "completed", "active client", "employed" and "family".

The fix, in housing: one written list every partner uses

Unsheltered street or places not meant for people

Emergency sheltered Agency A's "housed"

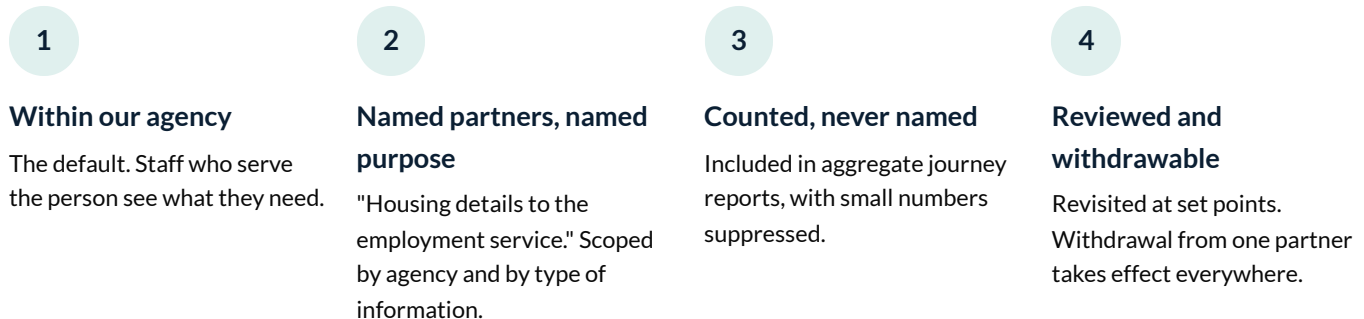
Provisionally accommodated Agency C's "housed"

At risk housed, but precariously

Example: the Canadian Definition of Homelessness (Gaetz et al., 2012, revised 2017). Funders recognize it; some program directives carry their own.

A consent model that keeps up with the journey

Following someone across services means holding more of their story, for longer. Consent should be a history that changes as the journey does, not a checkbox signed once at intake.



WHY IT MATTERS TO THE PEOPLE YOU SERVE

Canadians concerned about the protection of their privacy **89%**



Have refused to give personal information **78%**



Office of the Privacy Commissioner of Canada, 2024-25 survey (n = 1,500)

Consent as a history, not a checkbox

Housing provider	profile, history, goals
Employment service	housing details only
Mental health	clinical notes stay here
Shelter	withdrawn by the person

Granted, renewed, scoped, then withdrawn for one partner: each change is added, nothing is overwritten. Fictional agencies.

Granular, revisitable consent is supported by the dynamic consent literature, though most of it comes from health research. Explaining why records connect, and letting people say no, is the trust condition every public-attitudes study we reviewed points to. Canada now has a national standard written for this sector: **CAN/DGSI 100-11:2025**, Data Governance for Community and Human Services, published in May 2025 as part of a broader series.

First Nations, Inuit and Métis data governance

When data is from or about Indigenous peoples, the community decides what a journey measure includes, who sees it and where it lives. The First Nations Information Governance Centre sets out the First Nations principles of OCAP®; Inuit Tapiriit Kanatami's National Inuit Strategy on Research (2018) and the Métis Nation Data Strategy (2025) set out their own. This report does not restate them; go to the source, and to the nation whose data it is.

Kaye et al. (2015) • Teare, Pictor and Kaye (2021) • Office of the Privacy Commissioner of Canada (2025) • Digital Governance Standards Institute (2025) • OCAP® is a registered trademark of the First Nations Information Governance Centre (FNIGC).

Stability is not one checkbox

A checkbox can be ticked while a life is coming apart, and left blank while it is coming together. Record the person's goals in their own words, their own rating over time, and a shared domain underneath so the results can still be counted.



Keep my apartment through the winter.

See my daughter every weekend.

Finish my language course by spring.

Goals in the person's words, rated 1 to 10 at intake, at six months and at twelve

.82

agreement between raters on Goal Attainment Scaling when the worker who knows the person writes the scale

1.92

relative risk of death for "poor" against "excellent" on one self-rated health question, across 22 cohorts

1.19

odds ratio for staying engaged when progress is fed back to the person and worker, 58 studies

1.64

higher odds of a response when a questionnaire is shorter, Cochrane review

Short, person-rated measures are valid and responsive; much of the evidence comes from health and child mental health settings, so test them in your own context. Recovery research points to five domains people name themselves: connectedness, hope, identity, meaning and empowerment.

FOUR HABITS THAT KEEP MEASUREMENT LIGHT



Capture once, carry forward

Details collected at the first door travel with the person, with consent. Nobody retells their story four times.



Use moments that already happen

Intake, case review, discharge and one follow-up. No separate survey schedule.



A few questions, asked the same way

Three or four person-rated questions beat a long instrument that only gets half filled in.



Give the data back

When staff and clients can see progress, recording it stops feeling like reporting.

Steenbeek et al. (2010) • DeSalvo et al. (2006) • de Jong et al. (2021) • Edwards et al. (2009) • Leamy et al. (2011)

Person-rated measures, and the standard instruments

Should you use a standard tool or the person's own goals? Both can sit together. A standard instrument gives comparability; person-rated goals give meaning. Here is what the evidence says about each.

.82 • .64

Goal Attainment Scaling agreement when the treating worker writes the scale, against independent raters (Steenbeek et al., 2010)

2.45

points: the change threshold on the 0 to 10 Goal-Based Outcomes scale (Edbrooke-Childs et al., 2015)

1.4

responsiveness of a patient-generated measure, against under 0.45 for a standard health survey (Paterson, 1996)

.62 to .64

correlation of one life-satisfaction item with the five-item scale, 16,653 people (Cheung and Lucas, 2014)

Averaging 1 to 10 ratings assumes an interval scale they do not have, so report distributions or medians, not means (Krasny-Pacini et al., 2016). And length matters: completion of one short measure fell from 100 to 81.5 to 59.7 percent across three time points (Czachowski et al., 2011).

THE COMMON SECTOR INSTRUMENTS

Recovery Star

"Cannot be recommended as a routine clinical outcome tool": interrater reliability was inadequate (Killaspy et al., 2012). Fee-bearing, with mandatory training.

Self-Sufficiency Matrix

Validated against professional allocation decisions, not client outcomes (Lauriks et al., 2014); factor structure inconsistent across studies (Cummings and Brown, 2019).

VI-SPDAT

Total score did not predict return to services in 1,495 cases (Brown et al., 2018). The developer withdrew support in January 2021.

CHECK THE LICENCE BEFORE YOU BUILD IT IN

Outcomes Star family

Fee-bearing, with mandatory training

Outcome Rating Scale

Free licence is paper only; digital use needs a paid licence

Goal-Based Outcomes

Creative Commons BY-NC-SA; confirm terms for commercial platforms

Person, case, event: one journey on one screen

Any system that can show movement needs the same three building blocks. The examples here are designs in Mareto, with a fictional person and fictional agencies.

Person

One record per human being, however many programs or agencies they touch.

Case

Each program connection is a chapter, with its own dates, team and exit. A referral or an inquiry opens it; enrolment is a status, not the start.

Event

Every contact, assessment, consent change and follow-up, on one timeline.

One person, four programs, four agencies, eighteen months



■ Crisis ■ Stabilization ■ Recovery ■ No program attached

Jan 08 Shelter · emergency stay

Crisis

Mar 21 *No program for 40 days: the handoff nobody owns*

Apr 30 Community mental health · counselling

Stabilization

Jul 15 Employment service · job search

Stabilization

Aug 22 Shelter · return stay, on the same record

Crisis

Dec 02 Housing provider · lease signed

Recovery

The return in August lands on the same record, so the system learns from it instead of counting a new client.

Program view

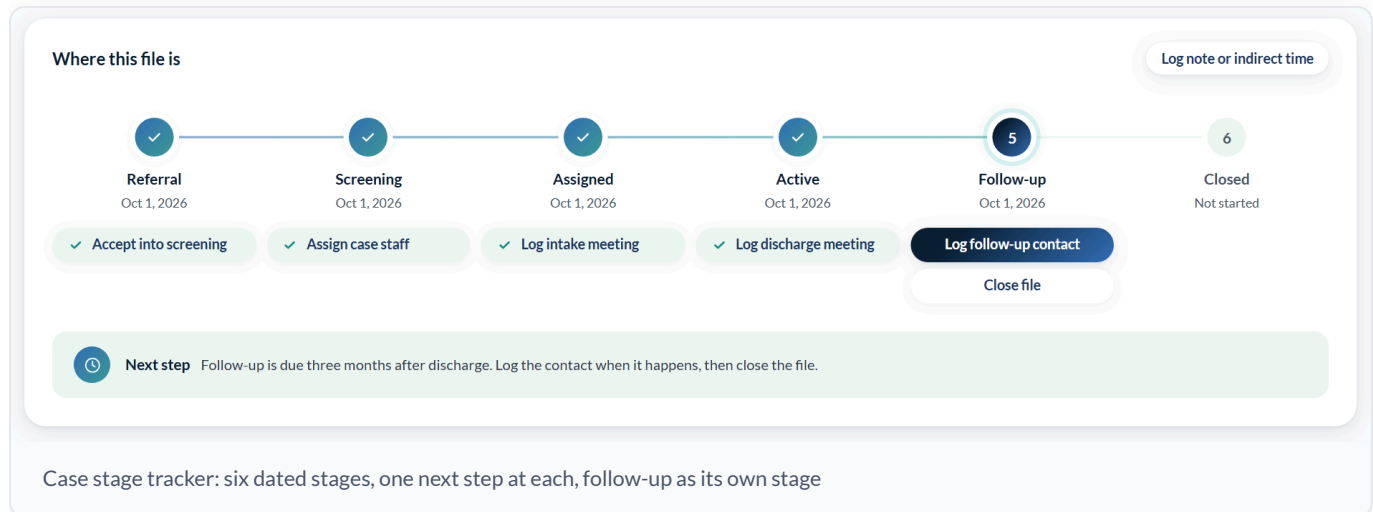
Caseloads, contacts and contract reporting, exactly as funders ask for them today. Program managers keep their view.

Journey view

The same people, read across programs and over time, with the five measures of movement. No second system, and no double entry.

What this looks like in practice

These screens come from a counselling program HelpSeeker is implementing now on Mareto, our case management platform. Staff appear as Counsellor A to H and no client records are shown. The first screen tracks one file through every stage.



DESIGN CHOICE

Every stage is dated

Referral, screening, assigned, active, follow-up, closed. Dates on each change are what make it possible to count moves, not just states.

DESIGN CHOICE

A logged event moves the file

Each stage has one next step: accept into screening, assign staff, log intake, log discharge. Nobody has to remember to update a status box.

DESIGN CHOICE

Follow-up has a clock

Follow-up had been informal. It is now a stage, due three months after discharge, so it gets done and gets counted.

WHAT WE LEARNED BUILDING IT

Agree the stages first

The stages were drafted with program managers before anything was built. The definitions are the work; the dashboard is the easy part.

Clean up roles

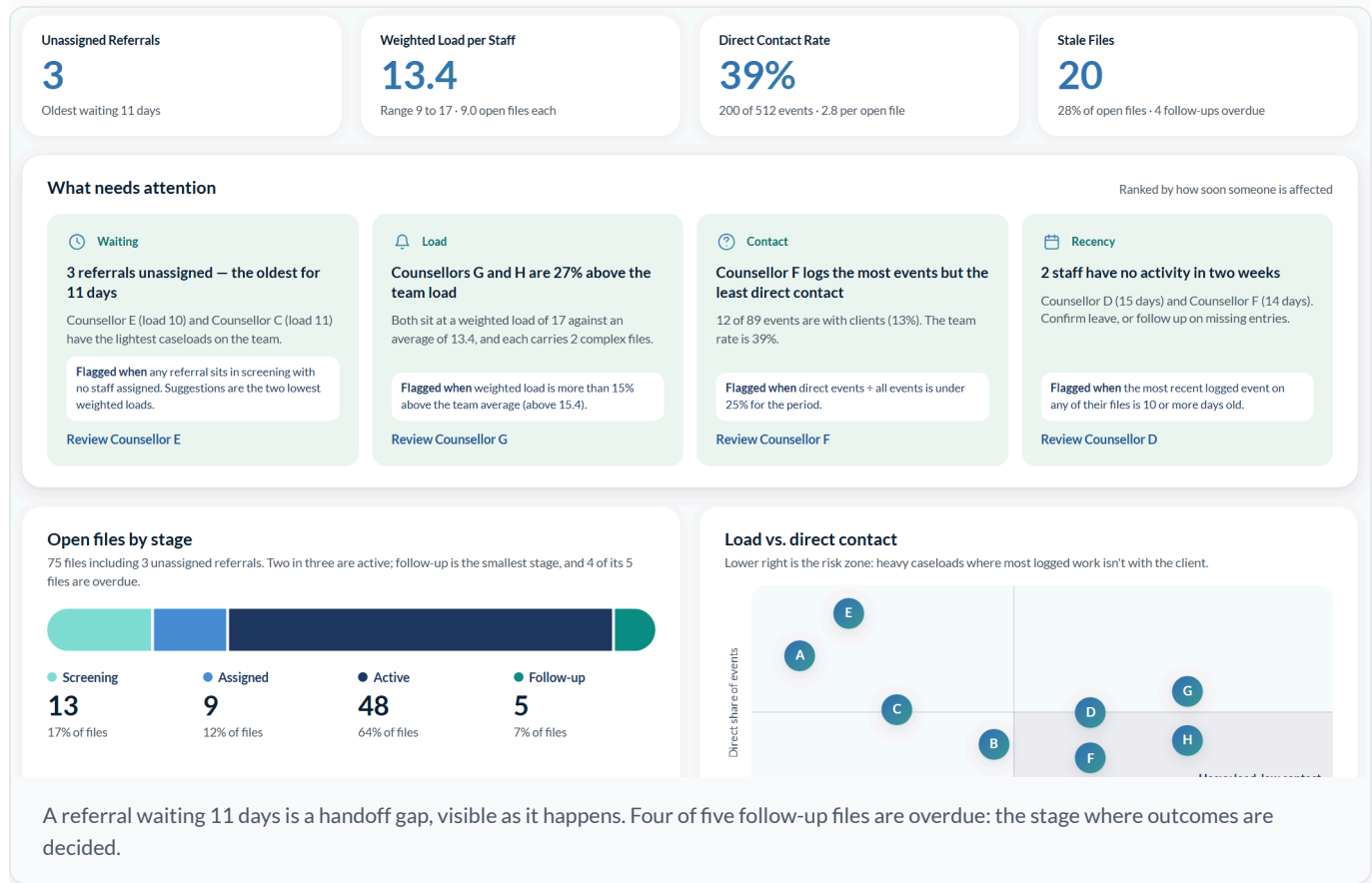
Administrative staff had been added to every file, so they looked like counsellors with huge caseloads. Access now follows role, not a name on the file.

One ID, cleaned once

Legacy client numbers in two formats confused staff. One person ID, cleaned at migration, is the foundation every view rests on.

A caseload view a manager can act on today

The second screen reads across every file in the program. It ranks what needs attention by how soon a person is affected, and every number says how it is computed.



WHAT WE LEARNED BUILDING IT, CONTINUED

Goals need categories

Goals stay in the client's words, mapped to shared parent categories so they can be counted across the program.

Make numbers explainable

Every tile carries its rule ("flagged when..."). A manager who cannot explain a number will not use it.

Support does not stop at launch

Weekly check-ins through the first months, and a screenshot with every piece of staff feedback, kept problems small.

Where are you today, and what is the next step?

A self-check, not a validated scale. In our experience most organizations start at level 1 or 2, and moving up one level is a realistic goal for a year.

- 1 **Counting activity**
Contacts, sessions and visits. Reports describe what the program did.
- 2 **Program outcomes**
Change measured between entry and exit, inside each program on its own.
- 3 **The person across your own programs**
One record per person in your organization. Internal handoffs and returns are visible.
- 4 **The person across partners**
Shared definitions and consent-based sharing. Handoff gaps and follow-up are measured.
- 5 **System learning**
Agencies and funders plan from which sequences of support lead to stability that lasts.

START ON MONDAY

Map one real journey. One person, with consent, across every service you know they touched. The gaps appear immediately.

Agree one definition with one partner, and write it down.

Reread your consent form. Can someone withdraw from one partner only?

Add one follow-up at three or six months after discharge. In continuing care, structured follow-up improved outcomes across 19 controlled trials.

IF YOU FUND OR PLAN A SYSTEM

Ask for movement

Add one journey question to reporting: where did people go next, and did it hold?

Set definitions once

Publish the definitions every funded agency uses, so the numbers can be added up.

Fund the follow-up

Recovery hold is measured only if someone is paid to check in after discharge.

Report in aggregate

Journey reports roll up to groups with small numbers suppressed.

Blodgett et al. (2014) • Dennis and Scott (2012). Some US federal substance use grants already require a six-month follow-up on every client, with 80 percent completion; completion tracks resourcing.

Follow-up after discharge changes outcomes

Recovery hold is only measured if someone checks in after the file closes. The evidence that this check-in also helps people is strong in continuing care and psychiatric transitions.

Proven

Continuing care effect, 19 controlled trials

At the end of care  .19

At follow-up  .27

Blodgett et al. (2014).

Proven

Abstinent days over four years

Quarterly recovery check-ups  1,026

Without check-ups  932

Dennis and Scott (2012).

0.88 • 0.91

odds of readmission with outpatient follow-up within 30 days, schizophrenia and bipolar disorder, 71,776 records (Marcus et al., 2017)

13.6 to 37.0

percentage points fewer early readmissions with transitional interventions, in the significant studies (Vigod et al., 2013)

80%

minimum six-month follow-up completion that some US federal substance use grants require on every client (SAMHSA)

Follow-up completion is achievable when someone pays for it

At Home/Chez Soi, Housing First, 24 months  89%

At Home/Chez Soi, usual care, 24 months  77%

Critical Time Intervention trial, 18 months  63.3%

Goering et al. (2014), Table A1; Baumgartner and Herman (2012). No trial of a three or six-month follow-up in community social services was located, so for those services this is our practice, with a strong analogue.

THREE THINGS TO TAKE AWAY

Measure people, not just programs

Movement shows when you follow the person and count the moves between phases.

Build the foundation first

One person ID, shared definitions and consent that lasts. Every measure depends on them.

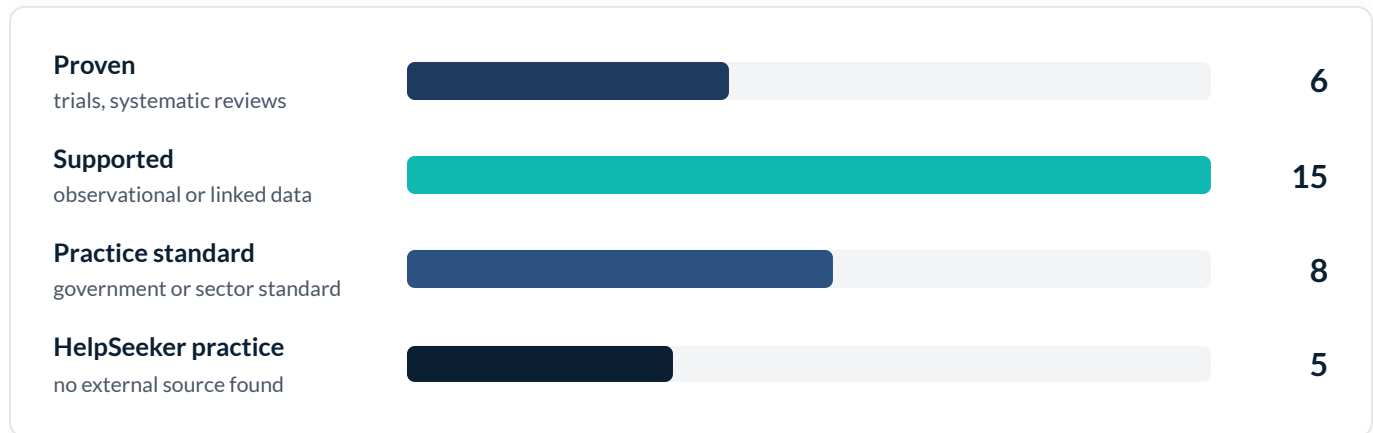
Let the person define stability

Their words and ratings, with a shared domain underneath so it still counts.

THE EVIDENCE BEHIND THIS REPORT

What the evidence says, at a glance

Before the webinar we graded every claim in it against the published literature and Canadian standards. Of 34 claims, 29 rest on external evidence; the rest are labelled as our practice.



FOUR THINGS THE EVIDENCE CHANGED IN OUR OWN THINKING

Warm handoffs are contested

The largest study, 2,690 referrals, found no effect on attendance; smaller studies found two to three times the attendance. The evidence that handoffs are dangerous is clearer than the evidence on how to do them.

Shared definitions are recognized, not adopted

Funders recognize published definitions, but their own program directives often write their own. Agreeing definitions locally still matters.

The national standard is new and specific

CAN/DGSI 100-11 was published in 2025 for community and human services, as one part of a data governance series.

OCAP® is First Nations-specific

FNIGC describes OCAP® as First Nations principles. Inuit and Métis governments set their own.

THE STRONGEST FINDINGS

Transitions between services are documented hazards. Time-limited bridging support across a transition reduces harm. Structured follow-up after discharge improves outcomes. And linked, person-level data shows patterns that no single program's report can.

The full evidence base, with every source checked on October 5, 2026, is in the technical paper behind this report, available on request. The published evidence is strongest in health, mental health and housing; settlement, disability and youth services have less journey research, which is one more reason to measure locally.

Seeing the journey, in your own programs

HelpSeeker works with community organizations, municipalities and funders across Canada to do the work in this report: mapping how people actually move through local services, agreeing the definitions partners will share, designing records backward from the reports organizations owe, and building in the follow-up that shows whether change lasted.

The examples in this report come from **Mareto**, our case management platform, built around one person, the cases they open in each program, and the events that happen along the way. Mareto is hosted in Canada and each organization owns its data. The ideas here work in any system; we are happy to talk through your own.

Talk to us: mareto.ca • helpseeker.org



NEXT WEBINAR

One client, four funders

Reporting once and
answering everyone

Thursday, October 29,
2026

11:00 AM Mountain •
60 minutes • free

How to design one set of
records that answers
every funder's report,
without double entry.
Scan to register.



KEY SOURCES

The research behind the five tools

The sources cited in this report. Every figure in this report traces to a source checked on October 5, 2026; the full list of 84 verified sources is available on request.

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